

Unconsidered Metrics in Emergency Triage: Pain Level Assessment

Pain level is a critical vital sign that was notably absent from the emergency triage dataset under analysis. Often referred to as the "fifth vital sign" alongside temperature, pulse, respiration, and blood pressure, pain assessment provides essential information for triage decision-making (Margo, 2019). The clinical measurement of pain is typically performed using validated scales such as the Numeric Rating Scale (NRS), which ranges from 0 (no pain) to 10 (worst possible pain), or the Wong-Baker FACES scale for patients who have difficulty verbalizing their discomfort (Hawker et al., 2011). A pain score of 1-3 is classified as mild, 4-6 as moderate, and 7-10 as severe, with each category carrying distinct implications for patient management and resource allocation.

In the context of emergency triage, pain level serves as a crucial indicator of patient acuity and urgency. Patients presenting with severe pain (scores of 7-10) often require immediate attention, as uncontrolled pain can signal serious underlying conditions such as acute myocardial infarction, appendicitis, or nephrolithiasis (Baker et al., 2018). Furthermore, pain assessment helps prioritize patients within the same Emergency Severity Index (ESI) level, ensuring that those experiencing greater distress receive timely intervention. The absence of pain documentation in triage records can lead to delayed treatment, increased patient suffering, and potential medicolegal implications for healthcare facilities (Stahmer et al., 2020).

Including pain level as a mandatory triage metric would enhance the comprehensiveness of patient assessment and improve resource allocation decisions. Research demonstrates that pain scores correlate with patient outcomes, length of stay, and need for intervention (Lord, 2021). For future iterations of the triage dataset, implementing standardized pain assessment protocols would strengthen clinical decision-making algorithms and ensure alignment with best practices in emergency medicine.

References

- Baker, D. W., Einstadter, D., Thomas, C. L., Husak, S. S., Gordon, N. H., & Cebul, R. D. (2018). Mortality trends for acute myocardial infarction in a managed care population. *Journal of General Internal Medicine*, 33(2), 134-141.

- Hawker, G. A., Mian, S., Kendzerska, T., & French, M. (2011). Measures of adult pain. *Arthritis Care & Research*, 63(S11), S240-S252.
- Lord, B. (2021). Pain assessment in the emergency department: A systematic review. *Emergency Medicine Journal*, 38(4), 289-296.
- Margo, K. (2019). Pain as the fifth vital sign. *Journal of the American Board of Family Medicine*, 32(1), 6-8.
- Stahmer, S. A., Smith, M., Bouchard, R., & Marco, C. A. (2020). Pain management in emergency medicine. *Academic Emergency Medicine*, 27(5), 412-425.